

MEDICAL RECORD - CONFIDENTIAL

PATIENT DEMOGRAPHICS

Patient Name: James Robert Anderson
Date of Birth: March 15, 1962 (Age: 62 years)
Sex: Male
MRN: 8472910
Date of Encounter: July 5, 2026
Encounter Type: Emergency Department Visit
Attending Physician: Dr. Sarah Mitchell, MD
Primary Care Provider: Dr. Michael Chen, MD
Insurance: Medicare Part A & B / Supplemental Plan G

CHIEF COMPLAINT

Acute onset chest pressure with radiation to left arm and jaw, associated with shortness of breath and diaphoresis, duration 45 minutes prior to arrival.

HISTORY OF PRESENT ILLNESS

Mr. Anderson is a 62-year-old male who presents to the Emergency Department with acute onset of substernal chest pressure beginning approximately 45 minutes prior to arrival while at rest watching television. The pain is described as a "heavy pressure" rated 8/10 in severity, radiating to the left arm, neck, and jaw. Associated symptoms include shortness of breath, profuse diaphoresis, nausea, and a sense of impending doom. He denies palpitations, syncope, or fever. He took one sublingual nitroglycerin tablet (0.4 mg) at home with minimal relief. EMS obtained ECG en route showing ST-segment elevation in leads II, III, and aVF. Patient received aspirin 325 mg chewed, nitroglycerin paste, and morphine 2 mg IV by EMS with partial pain relief (now 4/10). No prior similar episodes. No recent trauma, surgery, or immobilization. No recent travel.

PAST MEDICAL HISTORY

1. Hypertension (diagnosed 2010) - on lisinopril 20 mg daily
2. Type 2 Diabetes Mellitus (diagnosed 2014) - HbA1c 7.2% (last checked 2025-11), on metformin 1000 mg BID
3. Hyperlipidemia (diagnosed 2012) - on atorvastatin 40 mg daily
4. Chronic Kidney Disease Stage 3a (eGFR 52 mL/min/1.73m2) - nephrology follow-up annually
5. Obstructive Sleep Apnea - CPAP compliant
6. Remote history of tobacco use - quit 2015 (30 pack-year history)
7. No prior MI, PCI, CABG, stroke, or peripheral artery disease
8. No known coronary artery disease previously documented

SURGICAL HISTORY

1. Laparoscopic cholecystectomy (2018) - uncomplicated
2. Left knee arthroscopy for meniscal tear (2005)
3. No prior cardiac procedures

CURRENT MEDICATIONS

1. Lisinopril 20 mg PO daily
2. Metformin 1000 mg PO BID
3. Atorvastatin 40 mg PO daily
4. Aspirin 81 mg PO daily (taken this morning prior to event)
5. Tamsulosin 0.4 mg PO daily (for BPH)

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6. Vitamin D3 2000 IU PO daily
7. Omeprazole 20 mg PO daily (PRN for GERD)
Allergies: NKDA (No Known Drug Allergies)

SOCIAL HISTORY

Tobacco: Former smoker, quit 2015 (30 pack-year history)
Alcohol: Occasional wine with dinner (2-3 glasses/week)
Illicit Drugs: Denies
Occupation: Retired mechanical engineer
Living Situation: Lives with wife in two-story home
Exercise: Daily 30-minute walks, CPAP compliant nightly
Diet: Mediterranean-style, sodium-restricted

FAMILY HISTORY

Father: Deceased age 68 - MI
Mother: Deceased age 82 - stroke
Brother (age 65): Alive - HTN, hyperlipidemia, status post PCI 2020
Sister (age 58): Alive - T2DM
Two adult children: Alive and well
No family history of sudden cardiac death or cardiomyopathy

PHYSICAL EXAMINATION

Vital Signs: BP 148/92 mmHg (right arm), HR 98 bpm (regular), RR 22/min, Temp 98.4 F (36.9 C), SpO2 94% on room air, improved to 98% on 2L NC

General: Alert, oriented x3, in moderate distress, diaphoretic
HEENT: PERRLA, EOMI, moist mucous membranes, no JVD at 30 degrees
Neck: Supple, no carotid bruits, thyroid non-palpable
Cardiovascular: Regular rate and rhythm, S1/S2 normal, S4 present at apex, no murmurs/rubs/gallops, pulses 2+ bilaterally in carotids, radial, femoral, popliteal, DP/PT
Respiratory: Clear to auscultation bilaterally, no wheezes/rhonchi/rales, good air movement, no increased work of breathing
Abdomen: Soft, non-tender, non-distended, BS present x4 quadrants, no hepatosplenomegaly, no masses
Extremities: No edema, calves non-tender, no Homan's sign
Neurologic: Alert, oriented, CN II-XII grossly intact, motor/sensory intact, gait not assessed
Skin: Diaphoretic, warm, no rash/lesions

LABORATORY RESULTS

Troponin I (High Sensitivity): 12.4 ng/mL (ref: <0.04) - ELEVATED
Troponin I (repeat at 3 hours): 48.7 ng/mL - RISING
CK-MB: 28.5 ng/mL (ref: <6.3) - ELEVATED
CBC: WBC 11.2 K/uL, Hgb 13.8 g/dL, Hct 41.2%, Plt 285 K/uL
BMP: Na 139, K 4.2, Cl 102, CO2 24, BUN 28, Cr 1.4 (baseline 1.3), Glucose 186 mg/dL
Lipid Panel (fasting from 2025-11): Total 178, LDL 98, HDL 42, TG 190
HbA1c (2025-11): 7.2%
eGFR: 52 mL/min/1.73m2 (CKD-EPI)
BNP: 385 pg/mL (ref: <100) - ELEVATED
Coagulation: PT 13.2, INR 1.0, aPTT 28

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Lactate: 2.1 mmol/L (ref: 0.5-2.0) - SLIGHTLY ELEVATED
COVID-19 PCR: Negative
Influenza A/B: Negative

ECG FINDINGS

12-Lead ECG (07:23 AM): Sinus rhythm at 98 bpm. ST-segment elevation (2-3 mm) in leads II, III, aVF with reciprocal ST-depression in leads I, aVL, V2-V4. PR interval 168 ms, QRS 92 ms, QTc 440 ms. No pathologic Q waves. Comparison with prior ECG (2025-11): Previous ECG showed normal sinus rhythm, no ST-T changes.

Continuous Monitoring: Intermittent PVCs (3-4/min), no sustained arrhythmias.

IMAGING RESULTS

Chest X-Ray (Portable, 07:35 AM): Heart size at upper limits of normal (CT ratio 0.52). Mild pulmonary vascular congestion. No pleural effusions, no consolidation. Bilateral costophrenic angles clear.

Bedside Echocardiogram (08:15 AM): LVEF 45-50% (mildly reduced). Regional wall motion abnormality: Inferior and inferolateral hypokinesia. No pericardial effusion. RV systolic function normal. No valvular abnormalities. IVC dilated with <50% collapse.

ASSESSMENT / WORKING DIAGNOSIS

1. ST-Elevation Myocardial Infarction (STEMI) - Inferior wall, Occlusion likely RCA given STE II, III, aVF with reciprocal changes.
2. Acute Decompensated Heart Failure (new onset) - Elevated BNP, pulmonary congestion on CXR, reduced EF with inferior WMA.
3. Type 2 Diabetes Mellitus - Suboptimally controlled (HbA1c 7.2%).
4. Hypertension - Elevated on presentation (148/92).
5. Chronic Kidney Disease Stage 3a - Stable baseline.
6. Obstructive Sleep Apnea - On CPAP.

Plan:

1. EMERGENT CARDIAC CATHETERIZATION WITH PCI - Cath lab activated.
2. Dual antiplatelet therapy: Aspirin 325 mg + Ticagrelor 180 mg load.
3. Anticoagulation: Heparin bolus + infusion per protocol.
4. High-intensity statin: Atorvastatin 80 mg daily.
5. Beta-blocker: Metoprolol succinate 25 mg daily (hold if HR <60 or SBP <100).
6. ACE inhibitor: Continue lisinopril, consider up-titration post-PCI.
7. SGLT2 inhibitor: Consider empagliflozin post-stabilization for HFrEF + DM.
8. Diuretic: IV furosemide 20 mg for volume overload.
9. Glucose management: Insulin sliding scale, target 140-180.
10. Admit to CICU for monitoring, serial troponins, echo in 24-48h.
11. Cardiology consult (accepted). Nephrology consult for CKD dosing.
12. Smoking cessation counseling (reinforce). Cardiac rehab referral post-discharge.